

Patient Medical Record

Patient Information

Field	Detail
Full Name	Nguyen Van Minh
Date of Birth	March 14, 1978 (Age 48)
Sex	Male
Medical Record No.	MR-2026-04521
Occupation	Construction site supervisor
Marital Status	Married, 2 children
Address	District 7, Ho Chi Minh City
Admission Date	July 4, 2026
Attending Physician	Dr. Tran Thi Lan, Internal Medicine

Chief Complaint

Chest tightness and shortness of breath on exertion for the past 3 days.

History of Present Illness

The patient is a 48-year-old male with a known history of hypertension and type 2 diabetes mellitus who presented to the emergency department reporting intermittent retrosternal chest tightness, described as a pressure-like sensation, occurring with moderate physical exertion (climbing 2 flights of stairs) and relieved with rest. Episodes last 5-10 minutes. He also reports mild dyspnea on exertion and occasional diaphoresis during episodes. He denies radiation to the arm or jaw, denies syncope, and denies chest pain at rest. No fever, cough, or recent respiratory infection. He admits to poor medication adherence over the last 2 months due to work travel.

Past Medical History

- Hypertension, diagnosed 2019, on irregular treatment
- Type 2 Diabetes Mellitus, diagnosed 2021
- Dyslipidemia
- No prior myocardial infarction or stroke
- No known drug allergies

Family History

- Father: died of myocardial infarction at age 60
- Mother: hypertension, alive
- No family history of diabetes on maternal side

Social History

- Smoker: 1 pack/day for 20 years (still smoking)
- Alcohol: occasional, social drinking
- Diet: high in sodium and processed food due to work schedule
- Physical activity: sedentary, limited exercise

Current Medications (prior to admission)

- Amlodipine 5 mg once daily (irregular use)
- Metformin 500 mg twice daily
- Atorvastatin 20 mg once daily (irregular use)

Physical Examination

Parameter	Value
Blood Pressure	158/96 mmHg
Heart Rate	92 bpm, regular
Respiratory Rate	20 breaths/min
Temperature	37.0°C
SpO2	97% on room air
BMI	27.8 kg/m2 (overweight)

General: Alert, oriented, mild distress due to chest discomfort.

Cardiovascular: Regular rate and rhythm, no murmurs, no S3/S4, no jugular venous distension, peripheral pulses intact.

Respiratory: Clear breath sounds bilaterally, no wheezes or crackles.

Abdomen: Soft, non-tender, no organomegaly.

Extremities: No edema, no cyanosis.

Neurological: Grossly intact, no focal deficits.

Investigations

Electrocardiogram (ECG): Sinus rhythm, mild ST-segment depression in leads V4-V6, no acute ST-elevation.

Laboratory Results:

Test	Result	Reference Range
Fasting Blood Glucose	178 mg/dL	70-100 mg/dL
HbA1c	8.2%	<5.7% (normal)
Total Cholesterol	238 mg/dL	<200 mg/dL
LDL	162 mg/dL	<100 mg/dL

Troponin I (initial)	0.02 ng/mL	<0.04 ng/mL
Troponin I (6h repeat)	0.03 ng/mL	<0.04 ng/mL
Creatinine	1.0 mg/dL	0.7-1.3 mg/dL
Sodium	138 mmol/L	135-145 mmol/L
Potassium	4.2 mmol/L	3.5-5.0 mmol/L

Chest X-ray: No acute cardiopulmonary process; mild cardiomegaly.

Echocardiogram: Left ventricular ejection fraction 52%, mild left ventricular hypertrophy, no significant valvular disease.

Assessment

1. Unstable angina, ruled out for acute myocardial infarction - based on ECG changes with exertional symptoms and negative serial troponins.
2. Poorly controlled Type 2 Diabetes Mellitus - elevated HbA1c indicating suboptimal long-term glycemic control.
3. Uncontrolled Hypertension - likely due to medication non-adherence.
4. Dyslipidemia - elevated LDL despite statin therapy, likely related to non-adherence.
5. Tobacco use disorder - significant cardiovascular risk factor.

Plan

- Admit for observation and cardiac monitoring
- Start dual antiplatelet therapy (Aspirin 81 mg + Clopidogrel 75 mg daily)
- Initiate high-intensity statin (Atorvastatin 40 mg nightly)
- Beta-blocker (Bisoprolol 2.5 mg daily) for rate control and anti-anginal effect
- Adjust antihypertensive regimen: continue Amlodipine 5 mg, add Losartan 50 mg daily
- Optimize glycemic control: continue Metformin, consider adding a second oral agent; refer to endocrinology
- Schedule coronary angiography to evaluate for underlying coronary artery disease
- Smoking cessation counseling and referral to cessation program
- Dietary consultation for low-sodium, diabetic-friendly diet
- Patient education on medication adherence
- Follow-up in cardiology outpatient clinic in 2 weeks post-discharge

Discharge Instructions (anticipated)

- Strict adherence to prescribed medications
- Home blood pressure and blood glucose monitoring with a log
- Avoid strenuous activity until cleared by cardiology
- Return immediately if chest pain recurs at rest, worsens, or is accompanied by shortness of breath, sweating, or radiation to arm/jaw

This is a fictional case created for illustrative/educational purposes and does not represent a real patient.